

A Parent's Guide to Your Child's Evaluation

Evaluations, Counseling, and Parent Consultation for Children

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“My job is to figure out how you learn best.”

Why families seek an evaluation

Most families come to an evaluation with a question that has been building for a while. Reading isn't clicking the way it did for an older sibling. Homework that should take twenty minutes takes two hours. A bright child is suddenly refusing school, or a teacher has raised a concern at conferences, or the team has suggested testing and you want to understand what that means.

A psychoeducational evaluation exists to answer those questions. It is a structured way of understanding how your child thinks, learns, and manages the demands of school: their reasoning, memory, attention, academic skills, and social-emotional functioning. Just as important, it identifies what will actually help.

An evaluation is not a judgment of your child, and it is not a search for what is “wrong.” Children are more than any set of scores. A good evaluation describes a whole child: genuine strengths alongside genuine difficulties, and the connection between the two.

I evaluate children ages 5 to 16.

What an evaluation can tell you

Depending on the questions we set out to answer, an evaluation can address things like:

- Why reading, writing, or math is harder for your child than their effort would predict, including patterns consistent with specific learning disabilities such as dyslexia or dyscalculia
- How your child's attention, working memory, and executive functioning affect schoolwork, including patterns consistent with ADHD
- How anxiety, regulation, or social-communication differences, including autism, show up at school and at home
- What your child's genuine strengths are, and how to use them

- What supports, services, or accommodations make sense, described in the educational and functional terms that school teams use in their planning

Every evaluation ends with recommendations. I write them to be specific and realistic: things a teacher can do in a real classroom and things you can do at home, rather than a generic list.

What tests are used?

Every evaluation is built around your child's specific questions, so the exact instruments vary, but most comprehensive evaluations include measures across these areas:

- **Cognitive ability:** how your child reasons, solves problems, and processes information (for example, the WISC-V, the most widely used measure of children's cognitive abilities)
- **Academic achievement:** reading, writing, and math skills measured against grade and age expectations (for example, the WIAT-4), with deeper diagnostic measures of reading or math when those are the concern
- **Memory, attention, and executive functioning:** how your child holds and works with information, sustains focus, plans, and organizes
- **Social-emotional and behavioral functioning:** through standardized rating scales completed by you and your child's teachers (for example, the BASC-3), and measures specific to attention, anxiety, or social communication when relevant

These are the same standardized, nationally normed instruments used in school, clinic, and hospital settings. I'll review the planned battery with you at intake, and the written report explains what each measure assesses and what your child's results mean, not just the scores themselves.

One request: please don't prepare or practice with your child. There is nothing to study for, and coaching interferes with getting the accurate picture we're both after.

How the process works

1. A free phone consultation (15 minutes). You tell me what you're seeing and what you want to understand. I'll tell you honestly whether an evaluation is the right tool for your question and, if it isn't, what might be. If we move forward, you'll receive a written estimate of fees before anything begins.

2. Intake meeting and records review (weeks 1–2). We meet, parents only, to go through your child's history in depth. With your consent, I also review school records and gather input from the people who know your child: teachers, and any pediatricians, therapists, or other providers involved. Understanding a child fully means connecting with the professionals who work with them, and this input shapes which questions the testing needs to answer.

3. Testing sessions (weeks 2–3). Typically, your child and I meet for two or three sessions, depending on their age and stamina. More about what these are like below.

4. Feedback meeting and written report (weeks 4–6). First, we meet so I can walk you through the findings: what I learned, what it means, and what I recommend. You'll have time to ask everything you want to ask. The complete written report follows, and with your consent I can share it with your child's school and talk it through with the team.

Most families complete this process within four to six weeks of intake. If your timeline is shaped by an upcoming team meeting or school decision, tell me at the first call and I will do my best to plan around it.

What testing is actually like for your child

Parents often picture testing as something clinical or stressful. In practice, most children find it far more pleasant than they expected; many tell their parents it was fun.

A testing session is one adult and one child in a quiet room, working through a variety of activities: puzzles and patterns, questions and word games, memory challenges, reading, writing, and math tasks. Some activities feel like games. Some feel like schoolwork. They are designed to start easy and become challenging; every child reaches items they can't do, and that's built in on purpose. It's how the tests work, not a sign of failure.

I pace sessions around your child, with breaks and snacks as needed. I pay attention to what interests them and what they're good at, because children give their real effort when they feel comfortable and genuinely seen, and their real effort is what makes the results meaningful.

How to talk to your child about the evaluation

Keep it simple, honest, and low-pressure. What I say to children, and what you're welcome to borrow, is this:

“My job is to figure out how you learn best.”

That's the truth of it, in words a child can hold onto. Some other suggestions:

- **Do** call it “working with Dr. Hoitash” or “doing some activities and puzzles,” not “being tested.” For older children, it's fine to be more direct: “She helps figure out why some parts of school feel harder than they should, so we can make them easier.”
- **Do** tell them there are no grades and no passing or failing, and that nobody gets every item right; the activities are made that way.
- **Do** frame it around helping: “We want school to feel better, and this helps us figure out how.”

- **Don't** rehearse, drill, or prep: there is nothing to study for, and coaching only muddies the picture we're trying to see clearly.
- **Don't** promise specific outcomes ("this will get you extra time"). We don't know the answers yet; that's why we're asking the questions.

The day of testing: a normal breakfast, any regular medications as usual (unless we've discussed otherwise; please tell me about medications at intake), a reasonable bedtime the night before, and no big warnings in the car. Treat it like a normal, mildly interesting day.

After the evaluation: working with your child's school

An evaluation is most useful when everyone supporting your child is working from the same understanding. With your consent, I can:

- Share the report with your child's school team
- Consult with teachers and administrators about the findings and recommendations
- Attend IEP or 504 meetings with you

Having spent more than 20 years as a school psychologist in Massachusetts public schools, I know the IEP and 504 processes well: how teams gather information, make eligibility decisions, and build services. I write reports in the language those teams use in their planning. My approach with schools is collaborative: teachers and teams see your child every day, their perspective strengthens the evaluation, and the goal is a shared plan, not a contest of reports.

A note on terms you may hear: a private evaluation like this one is sometimes called an independent evaluation because it is conducted outside the school system. It can complement school testing, offer a second perspective, or take a broader look than the school's eligibility-focused assessment. If your situation involves disagreement with a school evaluation, we can discuss what role a private evaluation plays in that process during our consultation.

What happens next

For some families, the evaluation is the whole answer: they leave with an explanation and a plan, and the school takes it from there. For others, it points toward continued support. Two services are available if that is where things lead:

Counseling for children helps a child understand their own reactions and build strategies that work at school and at home. Regular parent meetings are a part of counseling.

Parent consultation is a standalone service for parents, with or without a child in counseling. Parents use it to make sense of a report or an IEP, to think through what keeps happening at home, or to decide what to do next.

Questions parents often ask

I was told to get a “neuropsych.” Is this the same thing?

“Neuropsychological evaluation” has become the catch-all term parents hear, but what a pediatrician or school is usually asking for is a comprehensive understanding of how your child learns, attends, and functions: cognitive abilities, memory, executive functioning, academic skills, and social-emotional functioning. That is exactly what my comprehensive psychoeducational evaluation covers, using the same standardized instruments. The situations that truly call for a neuropsychologist are those involving a medical or neurological condition, such as a brain injury, epilepsy, or a genetic or medical syndrome, where the question is about brain functioning in a medical context. If that describes your child, I'll tell you at our first phone call rather than proceeding.

Will my child be diagnosed with something?

Sometimes an evaluation identifies a pattern with a name, such as a specific learning disability like dyslexia, ADHD, autism, or anxiety, described in educational and functional terms. Sometimes it doesn't, and the answer is a clearer picture of how your child learns and what helps. Either outcome is useful. I do not make medical diagnoses and I do not prescribe or manage medication; those questions belong with your pediatrician or a psychiatrist, and my report is written to give them something useful to work from.

Will the results label my child?

The report belongs to you. Nothing is shared with anyone, including the school, without your written consent. And in my experience, children are usually relieved by what an evaluation gives them: an explanation. “Your brain learns reading differently, and here's what helps” is far kinder than the explanations children invent for themselves.

Should my child hear the results?

For most children, yes, in an age-appropriate way. Depending on your child's age and your preference, I can meet with them after the parent feedback session to explain what we learned, always leading with strengths. Children do better when they understand themselves.

What if my child is anxious or refuses?

Tell me beforehand. Between us, we can plan a warm-up visit, adjust pacing, or split testing into shorter sessions. I have spent many years working with anxious children, children with autism, and children for whom school has become a hard place. Meeting a child where they are is part of the work, not an obstacle to it.

What does it cost?

Evaluations are billed as a flat fee covering testing, scoring, the written report, and your feedback meeting. You'll receive a written Good Faith Estimate before we begin, as required by federal law. My practice is private pay and I am not in network with any insurance plan, so families should plan for all services to be an out-of-pocket

expense. After each service you'll receive an itemized receipt, which families may be able to use for HSA or FSA reimbursement; please confirm eligibility with your plan administrator. Payment plans are available, just ask.

About Dr. Hoitash

Rebecca Hoitash, PsyD, is a Licensed Educational Psychologist with more than 20 years of experience as a school psychologist in Massachusetts public schools, including 15 years in a specialized program for students with autism. She holds a doctorate in psychology from Rutgers University and a BA from Brandeis University, summa cum laude. Her practice, Hoitash Psychoeducational Services, provides psychoeducational evaluations, counseling, and parent consultation for children in Wellesley and the surrounding MetroWest communities.

Ready to talk it through?

The free 15-minute phone consultation is where we figure out whether an evaluation is the right next step for your child. There's no obligation, and if something else would serve you better, I'll tell you.

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